PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Addressing palliative care and end-of-life issues in patients with advanced cancer: a systematic review of communication interventions for physicians not specialized in palliative care.
AUTHORS	Harnischfeger, Nele; Rath, Hilke; Oechsle, Karin; Bergelt, Corinna

VERSION 1 – REVIEW

REVIEWER	Kyle Sue Memorial University of Newfoundland
REVIEW RETURNED	03-Jan-2022

GENERAL COMMENTS	Great study on a very important topic that surprisingly hadn't been previously addressed in such a review. Just a few minor spelling and grammar issues, as well as a few areas of clarification where I was uncertain. Page 4 of the PDF (page 3 of the article): line 29: grammar - "...should communicate early about issues..." line 49: grammar - "...more actively" Page 5 of PDF: line 20: "Previous" needs to be capitalized line 42: "Preferred" needs to be capitalized Page 7 of PDF: line 26 / line 53: "by consensus" instead of "by consent" Page 8 of PDF: line 29: "were excluded" instead of "where excluded" Page 15 of PDF: line 15/16: grammar - "..exclusively targeted physicians other than oncologists" Page 20 of PDF: line 17: I'm not certain what is meant by "transfer the contents". This needs to be rephrased or clarified. Page 21 of PDF: line 59: spelling - "efficacy" Page 22 of PDF: line 25: "...resp. interrupted time series" -- Is this a typo?

REVIEWER	Fahad Saeed University of Rochester Medical Center

REVIEW RETURNED	05-Feb-2022

GENERAL COMMENTS	I applaud the authors for undertaking a very important study. A few comments: 1. The article needs editing for language. This is not a big issue though. 2. It will be more useful if authors , based on their review, can add sections on the characteristics /design of the future intervention. 3. I would encourage the authors to add a section on scales/measurement methods for future studies. 4. I would encourage authors to comment on healthcare utilization and effects on physician burn out from the interventions they discuss,

VERSION 1 – AUTHOR RESPONSE

Reviewer #1

1. Page 4 of the PDF (page 3 of the article):
line 29: grammar - "...should communicate early about issues..."
line 49: grammar - "...more actively"

1. Authors' response: Thank you for pointing this out. We corrected the mistakes (please see page 3).

2. Page 5 of PDF:
line 20: "Previous" needs to be capitalized
line 42: "Preferred" needs to be capitalized

2. Authors' response: We corrected the mistakes (please see page 4).

3. Page 7 of PDF:
line 26 / line 53: "by consensus" instead of "by consent"

3. Authors' response: We corrected the mistakes (please see pages 6 and 7).

4. Page 8 of PDF:
line 29: "were excluded" instead of "where excluded"

4. Authors' response: We corrected the mistake (please see page 7).

5. Page 15 of PDF:
line 15/16: grammar - "..exclusively targeted physicians other than oncologists"

5. Authors' response: We corrected the mistake (please see page 8).

6. Page 20 of PDF:

line 17: I'm not certain what is meant by "transfer the contents". This needs to be rephrased or clarified.

6. *Authors' response:* We are thankful for this comment and agree that our wording was unclear. We rephrased the sentence (please see the last sentence on page 11).

7. Page 21 of PDF:
line 59: spelling - "efficacy"

7. *Authors' response:* We corrected the mistake (please see page 17).

8. Page 22 of PDF:
line 25: "...resp. interrupted time series" -- Is this a typo? I

8. *Authors' response:* Thank you for this comment. The term "interrupted time series" is the name of a study design, which is used like this in the risk of bias assessment tool, that we used. Nonetheless, the abbreviation "resp." was used incorrectly and we corrected it into a slash. The reason we use a slash here is that the study designs "cohort study" and "interrupted time series" are both rated medium in the assessment tool and thus are counted together (please see page 19).

Reviewer #2

1. The article needs editing for language. This is not a big issue though.

1. *Authors' response:* We thank the reviewer for this comment. We are sorry for some linguistic mistakes in our manuscript. We edited language and corrected spelling and grammar mistakes throughout the manuscript.

2. It will be more useful if authors, based on their review, can add sections on the characteristics /design of the future intervention.

2. Authors' response: We are thankful for this idea and added a section "implications" to the discussion part of our manuscript. There we added a subheading on the characteristics/the design of future interventions (please see page 22).

3. I would encourage the authors to add a section on scales/measurement methods for future studies.

3. *Authors' response:* We added a section on the design of future studies under the heading "implications" and discussed scales/measurement methods for future studies in there (please see pages 21-22).

4. I would encourage authors to comment on healthcare utilization and effects on physician burn out from the interventions they discuss.

4. *Authors' response:* We deliberately set a focus on communication-related outcome parameters, because we were mainly interested if interventions can effectively improve the physicians' communication behavior. However, we are thankful for the idea of commenting on some non-communication-related effects of the discussed interventions and added a brief description of the effect on healthcare utilization and physician burn out in the results part of our manuscript (please see page 17).